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avalon

June 16, 2025

Mehmet Oz, M.D.
Administrator, Centers for Medicare & Medicaid Services
U.S. Department of Health and Human Services
7500 Security Boulevard
Baltimore, MD 21244

Thomas Keane, M.D.
Assistant Secretary for Technology Policy (ASTP)
Office of the National Coordinator for Health Information Technology (ONC)
U.S. Department of Health and Human Services
330 C St SW, Floor 7
Washington, DC 20201

Submitted via

RE: Request for Information; Health Technology Ecosystem [CMS-0042-NC]

Dear Drs. Oz and Keane:
Avalon Healthcare Solutions ("Avalon") applauds CMS and ASTP/ONC's interest in using technology to transform the healthcare ecosystem. We commend the goal of creating a patient-centered, digitally enabled healthcare system that facilitates informed decision-making. As a leader in diagnostic intelligence, we are advancing CMS' efforts to ensure health information flows to where it is needed, rather than being locked away in silos. We appreciate the opportunity to provide our perspective on this issue.

As a clinical and information technology company dedicated to diagnostic laboratory services, Avalon brings a unique perspective to the questions in this RFI. Our work with health plans and lab providers is about making actionable, real-time lab data available to reduce unnecessary medical interventions and support evidence-based care decisions. We recognize the limitations of current data exchange practices and the need for policy improvements.

Laboratory testing is a foundational component of modern healthcare and influences approximately 70% of all medical decisions. With nearly 14 billion clinical laboratory tests conducted annually in the U.S., it is the most utilized medical benefit. Given the central role of laboratory testing and CMS's focus on making quality, actionable data available through modern technology, laboratory data must be a priority. CMS should recognize laboratory testing as the gateway to personalized care. Ensuring the interoperability and usability of diagnostic data – particularly genomic and biomarker results – is essential to timely, individualized care informed by the most precise clinical insights.

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Avalon is supportive of HHS' past efforts to advance interoperability and expand patient access to data. In our view, the most meaningful progress will come not just from making more data available, but from making the right data usable.

Avalon offers its expertise to make the following recommendations about three RFI questions:

- How important is it for all data in EHRs to be accessible for exchange, and what are the current challenges, technical barriers, and privacy or cost implications for making that data available?
- Should ASTP/ONC consider removing or revising any of the information blocking exceptions to further the access, exchange, and use of electronic health information (EHI) and to promote market competition?
- How can CMS encourage providers and payers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal?

1. **Mandate a universal standard for the transfer of all clinical information to improve the usability and reliability of data.** While the 21st Century Cures Act's API mandates create technical pathways for data exchange through FHIR and USCDI standards, they primarily address the avenues for data transmission rather than transmission of clinically meaningful data, which requires consistency in how laboratory data is recorded, transmitted, and interpreted. Current implementation varies in terminology mapping, value set adoption, and clinical context preservation, creating data that is technically accessible but clinically unreliable. CMS's Interoperability and Prior Authorization Final Rule reinforces this need by requiring standardized FHIR-based APIs, yet semantic interoperability challenges persist.

CMS and ASTP/ONC should mandate comprehensive semantic interoperability requirements, including required terminologies, standardized units of measure, and consistent reference ranges to transform fragmented data into actionable clinical intelligence. This comprehensive approach would ensure that laboratory results are interpreted consistently across all payers and providers, reducing medical errors and spending on inappropriate testing while facilitating seamless care coordination throughout a patient's entire health journey.

2. **Strengthen enforcement of information blocking provisions to ensure timely access to complete datasets.** Current information blocking enforcement allows penalties up to $1 million per violation, yet varying investigation procedures and response timeframes create uncertainty that undermines deterrent effects. Laboratory data blocking creates cascading clinical risks: delayed access to historical lab results can lead to duplicate testing, missed trending patterns that indicate disease progression, and inability to identify drug interactions based on

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recent lab values. When health plans cannot access comprehensive lab histories, they cannot effectively identify high-risk populations for targeted interventions or evaluate the appropriateness of requested diagnostic testing. At the same time, care teams lose visibility into critical testing trends needed for treatment optimization. CMS and ASTP/ONC should increase the penalty, enhance the detection, investigation, and prosecution of potential information blockers, and address these situations swiftly.

When stakeholders can reliably access complete laboratory datasets, they can deploy emerging technologies, such as predictive analytics for early disease detection and pharmacogenomic testing to optimize medication management. Market-driven solutions require comprehensive data access to deliver on the promise of precision medicine.

3. **Create a minimum security framework to protect patient data across all healthcare ecosystem entities handling PII/PHI**. As an entity that exchanges critical diagnostic information between providers and payers, Avalon recognizes that inconsistent security requirements create vulnerability gaps and undermine patient trust**.** While HIPAA establishes security requirements for covered entities and business associates, third-party applications accessing patient data through FHIR APIs, health information intermediaries, and digital health product developers operate under varying security standards. A standardized security framework would democratize access to healthcare data for vetted developers, shifting competitive focus from data acquisition challenges to delivering clinical value.

Avalon recognizes this requires striking a balance between privacy protections and innovation. While emerging technologies and digital health tools introduce novel privacy risks, future regulatory changes should not prevent the development of advanced technologies or the use of tools that can improve disease prevention and chronic disease management.

With laboratory data driving most medical decisions, addressing these foundational challenges will unlock the full potential of digital health technologies to improve patient outcomes and reduce healthcare costs for Medicare beneficiaries.

We appreciate CMS's leadership in advancing health technology. If you have any questions or would like to meet about our recommendations, please do not hesitate to contact Alex Sommer at alex.sommer@avalonhcs.com.

Sincerely,

DocuSigned by:
William L. Kerr
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Bill Kerr, M.D.
Chief Executive Officer
Avalon Healthcare Solutions